

WESTERN AUSTRALIA

STATE HAZARD PLAN

FOR

HUMAN EPIDEMIC

WESTPLAN – Human Epidemic

Prepared by

Department of Health, Government of Western Australia

APPROVED AT STATE EMERGENCY MANAGEMENT COMMITTEE MEETING

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Amendment list

AMENDMENT		DETAILS	AMENDED BY
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1	July 2008	Complete re-write	D. Mak
2	June 2010	Amendment to SHEC role	A. Robertson
3	April 2014	Routine revision	D. Mak
4	May 2016	Statement of fact changes	J. Heslop and SEMC Secretariat
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A copy of this Westplan is available on the State Emergency Management Committee (SEMC) internet site:

www.semc.wa.gov.au/resources/policies-and-plans/westplans

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1 INTRODUCTION

1.1 Aim and objectives

1.1.1 Aim

This Westplan – Human Epidemic details the State of Western Australia's strategic arrangements for the management of a human epidemic.

1.1.2 Objectives

The objectives of this plan are:

a) To define the emergency management structure and coordination arrangements for Government organisations, non-Government organisations and the private health sector to be utilised for the response to a human epidemic.
b) To provide the framework for emergency responses to a human epidemic at State, District and Local levels.
c) To provide guidelines for the operation of this Plan upon its activation of the response provisions and to identify the key actions that may need to be undertaken in the event of a human epidemic.
d) To identify and to specify the roles and responsibilities of Government organisations, non-Government organisations and the private health sector involved in the response to a human epidemic.

1.2 Scope

This Plan applies to all occurrences of human epidemics in Western Australia (WA) managed within the legislative responsibilities of the Department of Health (WA Health), and that require activation of State emergency arrangements.

1.3 Hazard definition

For the purpose of this plan, a human epidemic is the occurrence of more cases of an infectious or transmissible disease than would be expected in the State's population or a sub-group of the State's population during a given time period.

The transmissible nature of diseases that may cause a human epidemic means that special and immediate actions are required to limit the spread of disease from infected persons to the wider community.

1.4 Related documents

This document is to be read in conjunction with the following suite of State Emergency Management (EM) documents:

- *Emergency Management Act 2005* (EM Act);
- *Emergency Management Regulation 2006* (EM Regulation);

- State Emergency Management Policy (State EM Policy);
- State Emergency Management Plan (State EM Plan);
- Relevant State Hazard Specific Plans (Westplans);
- State Emergency Management Procedures (State EM Procedures);
- State Emergency Management Guidelines (State EM Guidelines); and
- State Emergency Management Glossary (State EM Glossary).

It should be noted that the State EM Procedures are divided into Prevention, Preparedness, Response and Recovery sections, with individual procedures referred to as 'State EM Prevention Procedure', 'State EM Preparedness Procedure', 'State EM Response Procedure' and 'State EM Recovery Procedure', as applicable.

Additional documents that are applicable include, but are not limited to:

a) National Health Security Arrangements
b) Australian Health Management Plan for Pandemic Influenza (AHMPPI)
c) WA State Health Emergency Response Plan (SHERP)
d) Westplan – Chemical, Biological, Radiological and Nuclear (CBRN)
e) Westplan – Terrorist Act
f) State Health Emergency Response Plan;
g) State Emergency Welfare Plan and its annexures on Reception and Registration and Reunification;
h) State Emergency Public Information Plan activated by the State Emergency Public Information Coordinator; and
i) State Emergency Telecommunication Plan.
j) Western Australian Health Management Plan for Pandemic Influenza (WAHMPPI)
k) Individual health service infectious disease emergency management plans (however titled)

1.5 Authority to plan

The State Emergency Management Committee (SEMC) is responsible for ensuring the preparation of Westplans it considers necessary *[Section 18(1) Emergency Management Act 2005].* Under 20(1)(a) *Emergency Management Act 2005* the SEMC has directed the responsibility to ensure the development and review of the Westplan - Human to the State Human Epidemic Controller within the WA Department of Health.

1.6 Plan responsibilities

The State Human Epidemic Controller (SHEC) is responsible for ensuring that this plan is in place, and for the development and review of this plan in consultation with key stakeholders.

1.7 Exercise and review periods

1.7.1 Exercising

This plan shall be exercised at least annually in accordance with State EM Policy Sections 4.8 and 4.9 and the State EM Plan section 4.7.

1.7.2 Review period

This plan is to be reviewed by the SHEC, as the Hazard Management Agency (HMA), every five years and after each activation of the plan, in accordance with State EM Policy Statement 1.5.

1.8 Roles and responsibilities

The following organisations, agencies and individuals have roles and responsibilities under this plan that are detailed in Appendix C.

a) *WA Health – State Communicable Disease Control Directorate*
b) *WA Health – State Human Epidemic Controller*
c) *WA Health – State Human Epidemic Technical Advisory Group*
d) *WA Health – State Health Coordinator*
e) *WA Health – Metropolitan and Regional Population Health Units*
f) *WA Health – Child and Adolescent Health Service*
g) *WA Health – Communicable Disease Control Directorate, Sentinel Practitioners' Network, WA*
h) *Australian Health Protection Principal Committee (AHPPC)*
i) *Communicable Diseases Network Australia*
j) *WA Health - Environmental Health Directorate*
k) *Hospitals – public and private*
l) *St John Ambulance Australia (WA) Inc*
m) *Royal Flying Doctor Service*
n) *NurseWest*
o) *Australian Red Cross Blood Service*
p) *PathWest and private pathology laboratories*
q) *WA Police*
r) *Local Government Authorities*
s) *Water Corporation*
t) *Department of Agriculture and Food (WA)*
u) *Public Transport Authority*
v) *Department for Child Protection and Family Support*
w) *Department of Education*

2 PREVENTION AND MITIGATION

2.1 Responsibility for prevention and/or mitigation

The SHEC, as the HMA for Human Epidemic, is responsible for ensuring development and implementation of prevention and mitigation strategies. These responsibilities are set out in Appendix C.

2.2 Legislation and codes

a) *Health Act 1911* (WA)
b) *Emergency Management Act 2005* (WA) (EM Act)
c) *Emergency Management Regulations* 2006 (WA) (EM Regulations)
d) *Quarantine Act 1908* (Commonwealth)
e) *National Health Security Act 2007* (Commonwealth)

2.3 Prevention strategies

WA Health maintains the following routine prevention and control programs that minimise the risk of a human epidemic by reducing infectious disease transmission:

a) Environmental health programs to minimise risk of disease transmission, such as those designed to ensure provision of safe food and water, and effective sewerage systems.
b) Immunisation programs against vaccine-preventable diseases including poliomyelitis, diphtheria, tetanus, whooping cough, bacterial meningitis, measles, mumps, rubella, influenza, rotavirus, pneumococcal disease, varicella and hepatitis B.
c) Vector control programs, which prevent the transmission of diseases including Ross River virus, Murray Valley encephalitis, Barmah Forrest virus, typhus and plague.
d) Surveillance systems such as the notifiable disease surveillance system, the Sentinel Practitioners' Network, WA and Emergency Department Sentinel Surveillance alert health authorities to cases and clusters of preventable diseases, and lead to the initiation of control activities.
e) Health promotion and education activities, targeting both health professionals and the public.
f) Collaboration with national and international health agencies on disease prevention and control activities, including border control measures and quarantine procedures.

2.4 Mitigation strategies

WA Health maintains the following routine mitigation programs that minimise the impact of a human epidemic by ensuring early and effective control of infectious disease cases and outbreaks:

a) Prevention strategies as outlined in 2.3 above.

b) Outbreak management, which involves the rapid organisation of scientific investigations and application of disease control methods, including isolation, quarantine, hospital infection control measures and provision of treatment and prophylaxis, as appropriate to the particular disease and outbreak scenario.

3 PREPAREDNESS

3.1 Responsibility for preparedness

The Department of Health is responsible for maintaining statewide strategic preparedness for a human epidemic.

3.2 Planning and arrangements

Westplan – Human Epidemic will be used to manage human epidemics at all levels (Local, District and State) throughout WA.

The planning and preparedness information detailed below is intended to provide general advice to assist organisations to ensure they are prepared for a human epidemic.

Communication strategies, internally and for the public, must be developed and tested by organisations to ensure efficient, effective and appropriate distribution of relevant information as described in State EM Policy Section 5.6, State EM Plan Section 5.3.1, and State Emergency Public Information Plan.

Internal emergency management plans are to be developed and based on:

a) best practice principles;
b) technical and scientific knowledge;
c) historical data and information; and
d) local knowledge and experience.

3.2.1 Special Needs Groups

The State EM Policy Section 4.6 and State EM Plan Section 4.6.1 outline the individuals and groups whose circumstances require special consideration in emergency management planning.

Additionally, the following groups are likely to be more susceptible to, and/or more vulnerable to the effects of, a human epidemic:

- people who are immunologically compromised,
- the very young,
- the very old,
- people from Aboriginal and culturally and linguistically diverse backgrounds,
- homeless people,
- people living in custodial or residential care settings, and
- other marginalised and/or disadvantaged people.

Depending on the nature and geographic location of the epidemic, other groups may also need special consideration.

Identification of vulnerable populations along with methods to access these populations in a timely fashion, to provide advice and support, are a critical part of the preparedness process.

The Cultural and Linguistically Diverse (CaLD) community within WA must also be considered so that information is communicated effectively across the State.

3.2.2 Resources

Workforce:

a) Each organisation is responsible for their workforce.
b) Management of the workforce, especially the health workforce, is critical. Strategies need to be established to protect staff from becoming infected and prevent fatigue from overwork.

Health service capacity:

a) Demand for health care services such as hospital beds, emergency departments, primary health care, pathology laboratories and medical transport is likely to increase. Strategies need to be established to ensure the most effective use of health service capacity.

Pharmaceuticals and medical equipment:

a) Depending on the nature of the epidemic, demand for specific medications, vaccines and medical equipment (e.g. personal protective equipment) may increase. Strategies need to be established to enable timely access and consistent supply.

3.2.3 Training

Training of health care workers in emergency management is an essential requirement for preparedness. Health care workers must know how to respond to an emergency and understand their role in that process. Health care workers may be required to undertake tasks outside their usual scope of practice; this may require provision of specific training. Dealing routinely with public health follow-up contact tracing and management of infectious disease outbreaks, gives public health workers a framework of experience for dealing with human epidemic emergencies.

3.3 Community education

Community education should be tailored to the specific disease/epidemic scenario. For example, preparedness for pandemic influenza may include

community education on hygiene, infection control, use of antiviral medications and vaccination.

3.4 Quarantine and closure arrangements

The SHEC, in consultation with local clinicians and public health officials, will decide if, and when, quarantine of persons and closure of places is required to reduce the risk of disease transmission.

3.5 Local and district hazard emergency management plans

Local and district emergency management plans for human epidemic are detailed in regional health disaster plans, and hospital pandemic and surge management plans.

3.6 WA border agreements

The WA Chief Quarantine Officer is responsible for coordinating border control arrangements for the public health response to a human epidemic, in conjunction with the Commonwealth Director of Human Quarantine.

3.7 Arrangement for assistance from other jurisdictions

3.7.1 Australian Government assistance

The provision of Australian Government physical assistance is dependent upon established criteria and requesting arrangements. All requests for Commonwealth physical assistance are to be made by the SHEC or State Health Coordinator (SHC) through the State Emergency Coordinator, in accordance with State EM Policy Section 5.10 and State EM Plan Section 5.6.

3.7.2 Interstate assistance

In the event that State resources are unable to cope with the magnitude or complexity of a human epidemic, the SHEC may request, via the State Emergency Coordinator, to seek interstate assistance from the Australian Health Protection Principal Committee (AHPPC), depending on the nature of assistance required, in accordance with State EM Policy Section 5.10 and State EM Plan Section 5.6.

3.7.3 Assistance from overseas

In the event that National resources are unable to cope with the magnitude or complexity of a human epidemic, Emergency Management Australia may request international assistance.

3.8 Arrangements for assistance to other jurisdictions

Requests for State assistance to interstate and international jurisdictions will be directed to the SHC, either from the Department of Health (Australian

Government) or through Emergency Management Australia. The SHC will seek approval for such requests from the Director General (WA Health) before assembling appropriate medical teams, or other staff and equipment, for deployment. The SHC will keep the State Emergency Coordinator informed of such requests via the State Emergency Coordination Group (SECG) Executive Officer.

4 RESPONSE

4.1 Responsibility for response

The SHEC, as the HMA for the response to a human epidemic, is responsible for all response activities. However, a human epidemic, which is known to be due to a terrorist attack or incident, will be controlled by WA Police, as the HMA, in accordance with section 4.8 of this plan.

4.2 Epidemic detection

Sources of detection and notification of the onset of the hazard include:

a) An increase in statutory notifications (from medical practitioners and /or laboratories) of a particular notifiable infectious disease may be recognised by staff of the CDCD.

b) An increase in statutory notifications (from medical practitioners and /or laboratories) of a particular notifiable infectious disease may be recognised by staff of a Metropolitan and/or Regional Population Health Unit.

c) An increase in the occurrence of a particular non-notifiable disease may be recognised through a non-statutory surveillance system, e.g. Sentinel Practitioners Network WA, or by an individual practitioner and reported to a Metropolitan and/or Regional Population Health Unit or the CDCD.

d) An increase in cases of an infectious disease or symptoms that could be due to an infectious disease may be reported by the public or by health care workers to WA Health.

e) An increase in notifications of a particular infectious disease may be recognised by another State/Territory reported to WA Health by the Communicable Diseases Network Australia.

f) The Communicable Diseases Network Australia may receive information of an increased number of cases of a particular infectious disease occurring overseas.

g) Notification of significant disease detection by the Department of Agriculture and Food (WA).

This information should be communicated to the SHEC. Dissemination of alerts and public information upon activation of this plan will be coordinated by Communications Directorate, WA Health, working under the direction of the SHEC, in consultation with the State Emergency Public Information Coordinator (SEPIC) as outlined in the State Emergency Public Information Plan.

4.3 Levels of response

In the event of a human epidemic, the emergency response will normally be activated in stages. If necessary, stages may be activated concurrently to accelerate the emergency response.

Level 1 – Alert

This stage is activated when WA Health receives notification of a human epidemic emergency. During this stage, the SHEC monitors the situation to determine if the epidemic can be dealt with at the local or district level, or if further action needs to be taken, as outlined in this Plan. Depending on circumstances, linkages with the Communicable Diseases Network Australia will also be important.

If required, the SHEC will consult with members of the State Human Epidemic Technical Advisory Group (SHETAG)**,** the SHC and the State Emergency Coordinator, and inform participating organisations of the potential need to become involved in the emergency response.

Level 2 – Standby

This stage is activated when the SHEC judges that the information received in the Alert Stage warrants preparatory activities in readiness for an emergency response. Depending on the situation, the SHEC may undertake the following actions:

a) Call meetings of the SHETAG to consider the situation, to identify additional committee members who should be called to assist, and to determine a possible response strategy. Committee members, or their delegates, need to be contactable on a 24-hour basis.

b) Place appropriate staff at the Public Health Emergency Operations Centre (PHEOC), Metropolitan and Regional Human Epidemic Coordination Centre(s)(HECCs) on stand-by. These staff members need to be contactable on a 24-hour basis.

c) Consult with the SHC and State Emergency Coordinator, and provide participating combat agencies with information about the potential emergency response required, allowing them to undertake the preparations necessary for their involvement.

d) Inform the Director General (WA Health) and the Minister for Health (WA) of potential resource implications of the response strategy.

e) Notify and consult with Communicable Diseases Network Australia and the AHPPC.

f) Co-opt staff where necessary.

Level 3 – Response

This stage is activated when an emergency response to a human epidemic is considered necessary by the SHEC. Depending on the situation, the SHEC may:

a) Convene meetings of the SHETAG and seek assistance from the SHC and State Emergency Coordinator as necessary.

b) Activate the Public Health Emergency Operations Centre (PHEOC) under the authority of the Director CDCD.

c) Request the activation of the State Health Incident Coordination Centre (SHICC) under the authority of the SHC if required.

d) Request liaison officers from participating combat agencies, as required,

e) Consult with the SEC to discuss possible activation of the State Emergency Coordination Group (SECG) to assist in the provision of a coordinated multi-agency response to, and recovery from, a human epidemic.

f) Deploy departmental resources and resources supplied by participating combat agencies as required. This may include the dispatch of disease control teams to the relevant area(s) where they may:

- Arrange the isolation and treatment of cases;
- Conduct the tracing, testing and possible quarantining of contacts;
- Administer vaccines and/or other treatments; and
- Advise on, and institute, infection control measures as indicated by the specific circumstances of the epidemic.

g) Seek the cooperation of local health service providers and general practitioners and provide information as appropriate.

h) Provide ongoing briefings to the Director General (WA Health), the SEC and the Minister for Health (WA) of the emergency response.

i) Issue media releases to address public concerns and to disseminate information on how to reduce the risk of infection, and what to do if infection is suspected.

Level 4 – Stand-down

When the SHEC determines that the emergency response is no longer required, a stand-down of the activities initiated in the previous stages will occur.

The SHEC will:

a) Ensure that arrangements for recovery are in place.
b) Advise the SHC and the State Emergency Coordinator.
c) Notify members of the SHETAG and the SECG (if established).
d) Notify all staff and relevant Emergency Management agencies.
e) Inform the Minister for Health (WA) and Director General (WA Health).
f) Issue media statements to address public concerns and to reinforce previous information on how to reduce the risk of infection, and what to do if infection is suspected.
g) Determine arrangements for debriefing and evaluation.

4.4 Activation of this plan

4.4.1 Level of response

The Department of Health routinely manages minor epidemics and disease outbreaks through its disease surveillance and reporting systems. These epidemics and outbreaks are not considered emergencies, but are consistent with the definition of Level 1 and 2 incidents, as outlined in State EM Plan Section5.1.5. Such incidents do not warrant activation of this plan.

Activation of this plan will only occur when an infectious or transmissible disease occurrence or threat will require resources that exceed the capacity of existing health services. Such an incident would be consistent with the definition of a level 3 incident.

On activation of the response stage of this plan, the SHEC, as the Incident Controller is responsible for making and communicating the declaration of a level 3 incident in accordance with State EM Response Procedure 2.

4.4.2 Triggers for declaration of Emergency Situation or State of Emergency

Declaration of Emergency Situation

In accordance with State EM Policy Section 5.3 and State EM Plan Section 5.2.3, the SHEC or the State Emergency Coordinator may declare an 'Emergency Situation' when the situation requires the use of additional powers provided under the *EM Act*.

Declaration of a State of Emergency

The Minister responsible for the *EM Act*, on the recommendation of the State Emergency Coordinator, may declare a 'State of Emergency' when the situation requires the use of additional powers provided under the *EM Act*.

4.5 Incident Management System

WA Health has adopted an incident command system based on the Australasian Interagency Incident Management System (AIIMS).

On activation of the response phase of Westplan – Human Epidemic, the SHEC will become the Incident Controller and establish the Public Health Emergency Operations Centre, which will be coordinated by the Director, CDCD. The SHEC will request the SHC to activate the SHICC to coordinate the provision of the hospital and clinical health service response to, and recovery from, a human epidemic, if required.

4.6 Hazard management structure/arrangements

4.6.1 Multi-agency management groups and triggers for activation

A State Emergency Coordination Group (SECG) is established during a state of emergency, or may be established where an emergency occurs or is imminent, at the request of the HMA, or on the SEC's own initiative, to assist in the provision of a strategic, coordinated multi-agency response to and recovery from the emergency. Additionally, if a level 3 incident occurs, the HMA must consult with the SEC to determine if a SECG should be established.

The SECG is established in accordance with State EM Policy Statement 5.4.7, State EM Plan Section 5.2.3 and State EM Response Procedure 4.

4.6.2 Coordination structure

The SHEC oversees both the SHICC and the PHEOC and is responsible for coordination of the response to a human epidemic at the State level.

Metropolitan and Regional Human Epidemic Coordination Centres (HECCs), based in metropolitan and regional Population Health Units, working under the control of the PHEOC, are responsible for coordination and implementation of the public health response to a human epidemic at the local level.

Hospital and clinical health services, and non-government health sector responses, will be coordinated by the SHC, in conjunction with the SHEC.

District Emergency Coordinators, in conjunction with Metropolitan and Regional HECCs, will be responsible for assisting the coordination of resources at the local level by coordinating the activities of the Operational Area Support Group (OASG).

4.7 Quarantine and closure arrangements

The SHEC, in consultation with local clinicians and public health officials, will decide if, and when, quarantine of persons and closure of places is required to reduce the risk of disease transmission. These measures will not be

implemented without considering the effectiveness and feasibility of less disruptive disease control measures.

If quarantine and closures are required, the establishment of an SECG will be requested by the SHEC to ensure a coordinated multi-agency approach to the evacuation and resettlement of displaced persons.

4.8 Function support plans

Activation of any support plan required during the response to a human epidemic is detailed in Section 4.10 of this plan.

In the event of a terrorist act resulting in a human epidemic, WA Police assumes control of the terrorist emergency and the Department of Health manages the epidemic as a combat agency. The Commissioner of Police, as the Hazard Management Agency for a terrorist act, remains responsible for the overall management of the terrorism emergency.

4.9 Public information and media management

Intense media and public interest can be anticipated during a human epidemic. The following actions will assist with the handling of the media and public inquiries.

Overall responsibility for the preparation of WA Health media statements and coordination of media inquiries during an emergency event lies with the SHEC, as the Incident Controller. Media statements are only to be made by persons authorised by the SHEC.

The State Emergency Public Information Plan sets out arrangements for the provision of additional media relations support to the Manager, Public Affairs, Department of Health, if required. In the event of a human epidemic, which is known to be due to a terrorist act, this will be done in consultation with WA Police.

4.10 Activation of other plans in support of this plan

The following plans may be activated in part or whole to support Westplan – Human Epidemic at any time:

- State Health Emergency Response Plan (SHERP);
- WA Health – Infectious Disease Emergency Management Plan;
- State Emergency Welfare Plan and its annexures on Reception and Registration and Reunification; and
- The State Emergency Public Information Plan

State Health Emergency Response Plan (SHERP)

Activation of the SHERP should be considered when the decision to activate Westplan – Human Epidemic is made. The SHERP provides a State-wide framework to be utilised by WA Health to coordinate health emergency planning and support services during an emergency, including a significant human epidemic emergency.

State Emergency Welfare Plan

This plan prescribes the arrangements adopted by the Department for Child Protection and Family Support (CPFS), for the provision of Welfare Support services during emergencies.

During a human epidemic, the State Emergency Welfare Plan may be activated to assist with the provision of welfare support services as detailed within the plan, including establishment of an emergency catering service, provision of temporary accommodation, provision of emergency clothing and personal requisites, the provision of personal support services, the implementation of the Register.Find.Reunite System, and the provision of financial assistance, to eligible persons affected by the emergency.

The SHEC will liaise with the State Welfare Coordinator to determine the status of the 'Register. Find. Reunite'. When informed that the 'Register. Find. Reunite' has been activated; the SHC will advise the SHICC Coordinator and Regional Health Disaster Coordinators (RHDCs), who will inform the Health Care Units of the contact details. This will enable Health Care Units to refer callers to the State Central Registry and Inquiry Centre.

These services may be extended to those individuals, their family household, and others who have been placed under home quarantine or isolation. The provision of these services will be prioritised by the CPFS. In the situation of a human epidemic associated with a high mortality or disability rate, the CPFS will prioritise its services towards the care of children and dependents of deceased or seriously ill individuals.

4.11 Financial arrangements for response

Generally, to ensure accountability for expenditure incurred, the organisation with operational control of any resource shall be responsible for payment of all related expenses associated with its operation during emergencies, unless other arrangements are established. Detailed information in relation to the financial responsibilities of participating organisations are outlined in State EM Policy Section 5.12 and State EM Plan Section 5.4

The SHEC may seek further resources or support from within the State, and, if necessary, Commonwealth assistance as detailed in the State EM Policy Section 5.10 and State EM Plan Section 5.6.

5 RECOVERY

5.1 Responsibility for recovery

Local Governments are responsible for managing recovery following an emergency affecting the community in their districts. Where recovery activities are beyond the capacity of the local community, State support may be provided through the State Recovery Coordinator as detailed in the State EM Policy Section 6 and State EM Plan Section 6.

5.2 Transition to recovery

The SHEC will ensure that a recovery plan is developed in conjunction with key stakeholders. The recovery plan will address issues such as:

- re-establishment of normal health services,
- school and work attendance that may have been interrupted during the epidemic, and
- the mental health of epidemic survivors.

6 POST-OPERATIONS ANALYSIS AND REVIEW

6.1 Stand Down and debriefs

At the end of the human epidemic emergency response when the "Stand Down" order has been issued, the SHEC is to arrange for operational debriefing of all health staff and other personnel and organisations involved in the response to a human epidemic. Outputs of these debrief sessions will be analysed for themes and content.

6.2 Final Report

A final report including priority action items and recommendations will be prepared for submission to the Chair of the WA Health Emergency Management Committee for tabling at SEMC by the Director General (WA Health), and the relevant SEMC subcommittees in accordance with State EM Policy Section 5.11 and State EM Plan Section 5.7.

This report is to identify facets of planning and response that were beneficial, and any problems or shortfalls relating to the provision of health emergency management support. The final report should incorporate inputs from all agencies involved in the response to the human epidemic.

7 APPENDICES

7.1 Appendix A - Distribution list

Westplan – Human Epidemic is distributed in electronic form. The latest version is available from the SEMC Secretariat website. The only hardcopy versions distributed are the library copies shown below. Addressees on this list will be advised by email when a new or amended version of the Westplan is posted on the SEMC Secretariat website.

- **Emergency Management Australia**
  Crisis Coordination Centre

  EMA Institute Library (2 copies)

- **State Government Ministers**
  Minister responsible for administration of the Emergency Management Act 2005

  Minister for Health

- **State Emergency Management Committee**
  All members

  SEMC Secretariat

  All subcommittee members

- **Organisations with responsibilities in this plan**
  Hazard Management Agency

  Combat Agencies

  Support Organisations

  Emergency Coordinators

  Other agencies with responsibilities under this plan

- **Library Deposits (bound copies with contact details removed)**
  National Library of Australia, Legal Deposits Unit (2 copies)

  State Library of Western Australia, Battye Library (2 copies)

7.2 Appendix B - Glossary of terms/acronyms

Terminology used throughout this document shall have the meaning as prescribed in either Section 3 of the *Emergency Management Act 2005* or as defined in the *State EM Glossary.*

The following additional definitions apply:

HUMAN EPIDEMIC – the occurrence of more cases of an infectious or transmissible disease than would be expected in the State's population or a sub-group of the State's population during a given time period.

HUMAN EPIDEMIC COORDINATION CENTRE (HECC) – Metropolitan and Regional Human Epidemic Coordination Centres (HECCs) are established upon activation of the standby phaseof this plan. Human Epidemic Coordination Centres are under the control of the Public Health Emergency Operations Centre (PHEOC), and are responsible for coordinating the public health response to the epidemic which includes, but is not limited to, disease surveillance, data management, and public health management of infected persons and their contacts. Metropolitan and Regional HECCs may be established at metropolitan Public Health Units and regional Population Health Units (these levels are comparable to the District Emergency Management level), as required.

ISOLATION – Separation of people known to have an infectious disease from other people, for the period of communicability, to prevent or limit the direct or indirect transmission of the infectious agent from those infected to those who are susceptible to infection or who may spread the agent to others.

PUBLIC HEALTH EMERGENCY OPERATIONS CENTRE (PHEOC) - The PHEOC, which is coordinated by the Director, CDCD, oversees the public health activities of the Metropolitan and Regional Human Epidemic Coordination Centres, including oversight of disease surveillance, data management, and public health management of infected persons, and supports the SHEC and the SHETAG.

QUARANTINE – Separation of healthy contacts of an infectious case from other people.

STATE EMERGENCY COORDINATION GROUP (SECG) – a State-level group that is established if a State of Emergency declaration, in an Emergency Situation Declaration, or may be established by the State Emergency Coordinator, at the request of, or in consultation with, the HMA, to assist in the provision of a coordinated multi-agency response to, and recovery from, the emergency.

STATE HEALTH COORDINATOR (SHC) – The SHC has the authority to command the coordinated use of all health resources within WA, for response to and recovery from, the impacts and effects of a major emergency or disaster situation. During a human epidemic, hospital and clinical health service, and non-public-health sector responses, will be coordinated by the SHC, in conjunction with the SHEC.

STATE HUMAN EPIDEMIC CONTROLLER (SHEC) – The State Health Human Epidemic Controller is the Chief Health Officer,. The SHEC is the HMA and Incident Controller for a human epidemic and is responsible for coordinating the emergency response.

STATE HUMAN EPIDEMIC TECHNICAL ADVISORY GROUP (SHETAG) – This group assists the SHEC with the management of human epidemics by providing expert technical and scientific advice regarding epidemic control.

STATE HEALTH INCIDENT COORDINATION CENTRE (SHICC) – This State-level centre, under the direction of the SHC, addresses strategic management of an incident/disaster as well as facilitating management of state-wide events. During a human epidemic, hospital, clinical health service, and non-public-health sector responses will be coordinated by the State Health Incident Coordination Centre, in conjunction with the State Human Epidemic Controller.

WA HEALTH – Department of Health, Western Australia.

7.3 Appendix C – Roles and Responsibilities

WA Health – State Communicable Disease Control Directorate

a. Role: To provide expert advice to the SHEC and assist in the public health response to an emergency caused by a human epidemic.

b. Responsibilities:

1) Develop, maintain and disseminate Westplan **–** Human Epidemic.
2) Establish and maintain the State Human Epidemic Technical Advisory Group.
3) Advise the Chief Health Officer when activation of the Westplan **–** Human Epidemic is required.
4) Establish and manage the Public Health Emergency Operations Centre which is responsible for coordinating the State public health response to the epidemic. Activities included in the public health response include, but are not limited to, disease surveillance, data management, and public health management of infected persons and their contacts.
5) Ensure effective public health response by public health units. This may require establishment and management of Metropolitan/Regional Human Epidemic Coordination Centres.
6) Coordinate public health activities of all participating organisations.
7) Ensure efficient and effective use of all relevant resources.
8) Provide staff for the PHEOC.
9) Provide timely information updates to the SHEC.

WA Health – State Human Epidemic Controller

The Chief Health Officer is the State Human Epidemic Controller and the Incident Controller

a. Roles:

1) To determine when activation of Westplan – Human Epidemic is required.
2) To control the Statewide emergency response to a human epidemic with the assistance of the SHC.
3) Determine when stand down is appropriate and coordinate stand down process.

b. Responsibilities:

1) Ensure that appropriate state-wide measures are in place for the prevention of, preparedness for, response to, and recovery from, human epidemics.
2) Provide State-wide policy direction, advice and assistance regarding epidemic control.
3) Chair the State Human Epidemic Technical Advisory Group
4) Provide timely information updates to the State Emergency Management Committee, WA Health Communications Directorate, Director General (WA Health) and Minister for Health (WA).
5) Provide timely information, in conjunction with WA Health Communications Directorate, to the public.
6) Provide timely information, in conjunction with the State Health Coordinator, to health service providers.
7) Arrange a debriefing of all participants and the preparation of a post-operation report in accordance with the State EM Policy 5.11 and State EM Plan Section 5.7.
8) Request additional staff resources where required.

WA Health – State Human Epidemic Technical Advisory Group

a. Role: To assist the SHEC and CDCD with the management of human epidemics, including the provision of expert advice, as required.

b. Responsibilities: Provide expert technical and scientific advice regarding epidemic control.

c. Membership:

1) State Human Epidemic Controller
2) State Health Coordinator
3) Director , CDCD
4) Director, Environmental Health Directorate
5) Representative/s of Metropolitan Public Health Units and Regional Population Health Units
6) Manager, Disaster Preparedness and Management Unit
7) Legal Advisor, WA Health Legal & Legislative Services
8) Representative, WA Health Communications Directorate
9) Microbiologist, PathWest Laboratory Medicine WA

10) Infectious Diseases Physician, Infections & Immunology Health Network

11) Department of Education

12) Representative from General Practice

13) Other technical experts, as required

WA Health – State Health Coordinator

a. Role: In conjunction with the SHEC, coordinate the provision of the hospital and clinical health service, and non-public-health service response to, and recovery from, a human epidemic.

b. Responsibilities:

1) Provide staff for and operate the SHICC.

2) Coordinate and facilitate communications with hospitals through the SHICC.

3) Coordinate the management of all hospital resources planned for and utilised under this plan.

4) Distribute timely information, provided by the SHICC, to hospitals and hospital-based health service providers.

WA Health – Metropolitan and Regional Population Health Units

a. Roles:

1) To coordinate the prevention of, preparedness for and the public health response to, human epidemics at the Health Service (metropolitan) or Regional (non-metropolitan) level (these levels are comparable to the District Emergency Management level).

2) To assist in the recovery from human epidemics at the Health Service (metropolitan) or Regional (non-metropolitan) level (these levels are comparable to the District Emergency Management level).

b. Responsibilities:

1) Ensure that appropriate measures are in place for the prevention of, preparedness for the public health response to, human epidemics in the Population Health Unit's Metropolitan Health Service or WA Country Health Service Region.

2) Establish, manage and provide staff for Metropolitan and Regional Human Epidemic Coordination Centres, using own staff +/- surge capacity staff as directed by the PHEOC.

3) Membership of the Metropolitan and Regional Human Epidemic Coordination Centres may include, but is not limited to:

i. the Population/Public Health Unit's public health medical officer/physician, public health nurses and administrative support staff,

ii. a community health nurse manager (only applies to regional Public Health Units), and

iii. an environmental health officer (employed by the Population Health Unit or Local Government).

4) Implement epidemic control strategies as directed by the SHEC.

5) Manage Public Health Unit, Community Health and other staff deployed to Human Epidemic Coordination Centres to provide the local level public health response to a human epidemic.

6) Establish and manage Human Epidemic Coordination Centres using surge capacity staff provided by NurseWest and other appropriate agencies, as directed by the SHEC.

7) Provide Community Health staff for the local level public health response to a human epidemic (relevant to Regional Population Health Units only).

8) Support the collection and management of epidemiological data.

9) Provide timely information updates to the State Human Epidemic Coordination Centre.

10) Ensure efficient and effective use of all relevant resources.

WA Health - Child and Adolescent Health Service

a. Roles:

1) To provide staff to support the prevention of, preparedness for, the response to, and recovery from, human epidemics at the local Health Service level (comparable to the Local Emergency Management level).

2) To coordinate and provide WA Health's primary health care response required in the Perth metropolitan area in response to a human epidemic, if and when requested by the SHEC. This may include, but is not limited to, mass vaccination clinics and home visiting of people in home isolation and quarantine.

b. Responsibilities:

1) Provide Community Health staff for staff for the local level public health response to a human epidemic, if and when requested by the SHEC.

2) Provide surge capacity for nurses to be deployed to areas of need within the WA Health under the coordination of NurseWest.

WA Health - Communicable Disease Control Directorate, Sentinel Practitioners' Network, WA

a. Role: To assist the CDCD with surveillance of infectious diseases in general practice settings.

b. Responsibilities:

1) Collect and report routine surveillance information for infectious diseases subject to enhanced surveillance to the CDCD.
2) Collect and report additional surveillance information relevant to the human epidemic to the CDCD, if and when required.

Australian Health Protection Principal Committee

a. Role: A national committee reporting to the Australian Health Ministers' Advisory Council that oversees the Communicable Diseases Network Australia. This committee provides national coordination of emergency operational activity in health responses to disasters and health protection issues of national significance, including epidemics.

Communicable Diseases Network Australia

a. Role: An inter-jurisdictional forum for development of national policy and coordination of national response to communicable disease threats. The Director, CDCD may seek advice and collegiate support from the group. The group provides expert technical and scientific advice regarding epidemic control.

WA Health - Environmental Health Directorate

a. Role: To manage the environmental health component of the public health response to a human epidemic, as required.

b. Responsibilities:

1) Water Safety (drinking & recreational waters) – provide advice on the interpretation of water sample results and the treatment options for water of unsuitable quality.
2) Food Safety – provide advice on food quality monitoring.
3) Human Waste – provide advice on the safe disposal of human and animal wastes and the establishment of emergency sanitation.
4) Advise on vermin/vector control.
5) Hazardous Materials – provide advice on the toxic properties of chemicals.

6) Provide guidance to Local Government Environmental Health Officers to manage local response.

7) To provide officers & resources where requested and approved.

Hospitals – public and private

a. Role: To assist with the provision of the medical response during a human epidemic, including treatment and isolation of infected persons.

b. Responsibilities:

1) Report confirmed or suspected epidemic cases to the CDCD or local Public/Population Health Units without delay.

2) Provide isolation/treatment facilities for infected individuals requiring hospitalisation.

3) Assist with quarantine measures as directed.

St John Ambulance Australia (Western Australia) Inc

a. Role: to provide road ambulance services during a human epidemic, as required.

b. Responsibilities:

1) Coordinate and provide ambulance services if required for the transport of infected individuals or other persons in the course of the human epidemic.

Royal Flying Doctor Service

a. Role: to provide aero-medical transport services during a human epidemic, as required.

b. Responsibilities:

1) Coordinate and provide aero-medical transport services if required for the transport of infected individuals or other persons in the course of the human epidemic.

NurseWest

a. Role: To centrally coordinate the resourcing and allocation of temporary nursing staff used to supplement staffing in the management of a human epidemic, as required.

b. Responsibilities:

1) Monitor and report nursing staff deficits across Western Australia as requested by the SHC.

2) Monitor and report on temporary nursing staff availabilities.

3) Manage the recruitment of temporary nurses to the public sector.

4) Manage and coordinate the deployment of temporary nursing staff across WA sites according to site needs and strategic resource allocation.

5) Communicate with the SHEC and the SHC regarding temporary nursing staffing requests and deployments during a human epidemic response.

Australian Red Cross Blood Service

a. Role: To assist with the provision of blood products required for the medical response during human epidemic emergencies as required.

b. Responsibilities:

1) Provide blood and blood products if required for the treatment of infected individuals.

2) Assess the transmissibility of the epidemic agent by blood transfusion, and take action to prevent transmission.

3) Provide specialist consultation on transfusion medicine if required.

PathWest and private pathology laboratories

a. Role: To assist with the collection and testing of specimens from humans and other relevant sources during the management of a human epidemic, as required.

b. Responsibilities:

1) Report confirmed or suspected cases to the CDCD without delay if an epidemic is anticipated, suspected, or in progress.

2) Provide diagnostic pathology services for human, animal and environmental samples as relevant to the human epidemic.

3) Facilitate communication with medical practitioners through the laboratory service network.

WA Police

a. Role: To assist the WA Health with the emergency response to a human epidemic, as required.

b. Responsibilities:

1) Provide Emergency Coordinator/s to assist the HMA in the provision of a coordinated response during the emergency.

2) Consider requests for police assistance from the SHEC including:

   i. Assisting with isolation, quarantine and evacuation, if required, to deal with a human epidemic.

   ii. Providing road traffic management where appropriate.

   iii. Assisting with communications, resources, and public information.

   iv. Providing a Liaison Officer to the State Human Epidemic Emergency Management Committee, if required.

   v. In the event of mass casualties, provide Disaster Victim Identification.

   vi. Maintaining public order where required.

Local Government Authorities

a. Role: To assist with the public health and/or emergency response to human epidemics. Each Local Government has the responsibility to provide local environmental health services, to control any exposure sources or activities in its district which constitute a public health hazard, and to coordinate local recovery activities.

b. Responsibilities:

1) Provide Environmental Health Officers to Metropolitan and Regional Human Epidemic Coordination Centres, as required.

2) Assist with the investigation of human epidemics.

3) Assist with monitoring of food safety.

4) Assist with the safe disposal of contaminated waste.

5) Assist with the control of vermin or insect infestations, including reservoir elimination programs.

6) Provide support with other local resources as requested by the SHEC or local State, Metropolitan or Regional Human Epidemic Control Centres.

Water Corporation

a. Role: To ensure the availability of safe drinking water and safe waste water disposal in the event of a water-borne epidemic.

b. Responsibilities:

1) Sample drinking water supplies for testing and provide alternative safe drinking water if needed.

2) Monitor the adequacy of waste water disposal and provide alternative safe methods if needed.

Department of Agriculture and Food (WA)

a. Role: To provide technical advice to the CDCD to assist with the management of infectious diseases that are transmissible from animals to humans.

b. Responsibilities:

1) Alert the CDCD to new, emerging or notifiable infectious diseases in agricultural stock which are potentially transmissible to humans.

2) Contain the spread of infectious diseases in animal stock which may be transmissible to humans, consistent with the Animal Health National Response arrangements and Westplan – Animal and Plant Biosecurity.

Public Transport Authority

a. Role: To assist with the provision of transport for infected persons, their contacts and health staff, as required.

b. Responsibilities:

1) Coordinate the use of public transport services at the request of the SHEC.

Department for Child Protection and Family Support

a. Role: To assist with the welfare response to a human epidemic. This may include, but is not limited to, assisting people under home isolation and home quarantine, and their dependents.

b. Responsibilities:

1) Assist with the provision of services to individuals, their family household and others affected by the emergency, as outlined in Westplan – Welfare.

2) Prioritise the care of children and dependents of deceased or seriously ill individuals.

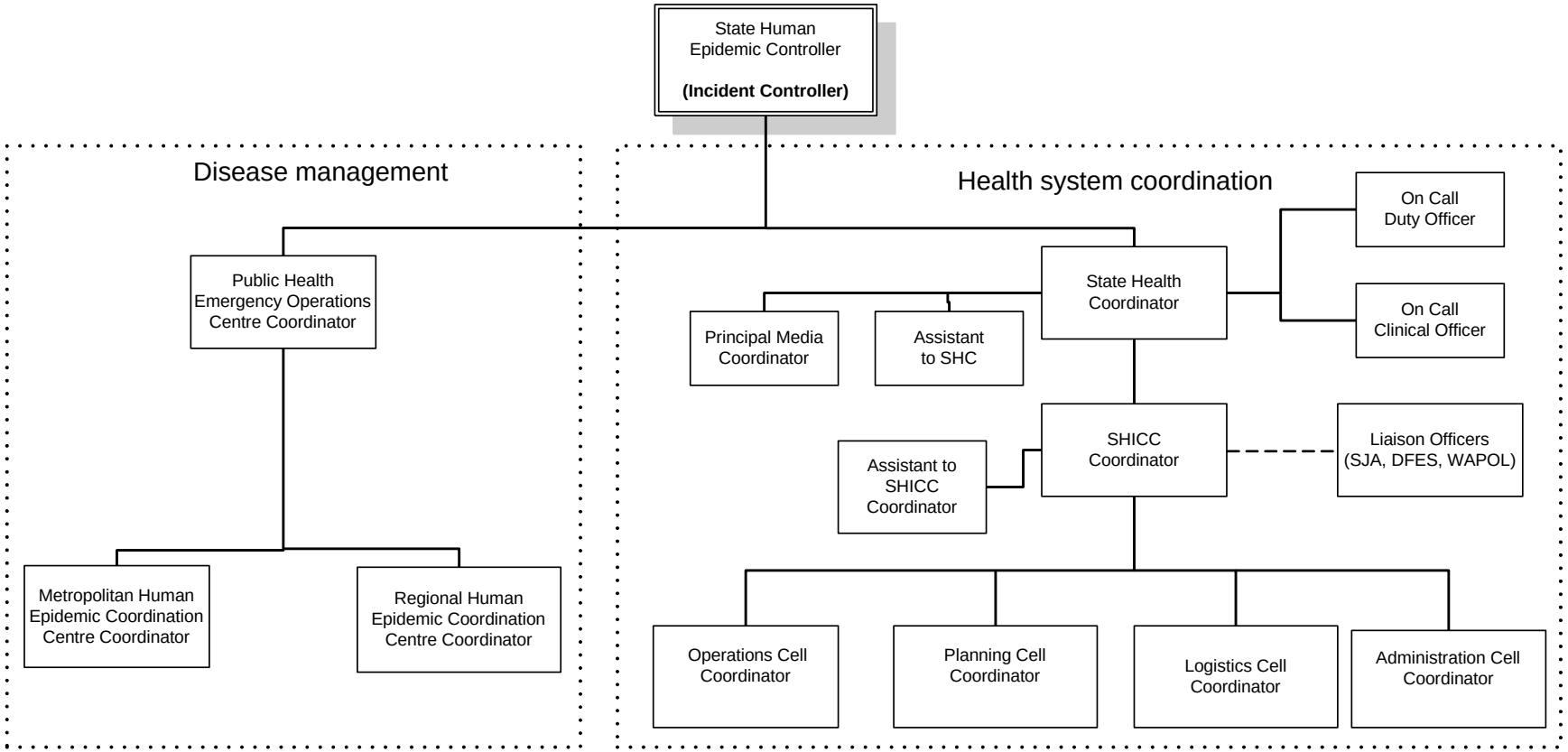
Department of Education

a. Role: To assist the WA Health with the emergency response to a human epidemic, as required.

b. Responsibilities:

1) Assist with the provision of information to school leaders, school staff, students and parents.
2) Assist with the provision of health services. This may include, but is not limited to use of school facilities for mass vaccination and medication distribution.
3) Support implementation of disease control measures such as home isolation and home quarantine.
4) To have an identified liaison person from the to enable a two-way communication process to be implemented with an identified Department of Health liaison person.
5) To Liaise with Catholic Education Office and the Association for Independent Schools in Western Australia to allow a collaborative education sector approach.

7.4 Appendix D – State Health Emergency Command and Control



7.5 Appendix E – Contact details for Regional Population Health Units and Metropolitan Public Health Units

Outside the Perth metropolitan area

• Coastal and Wheatbelt (Northam)	(08) 9622 4320,	0429 784 073*
• Goldfields (Kalgoorlie)	(08) 9080 8200,	0419 906 024*
• Great Southern (Albany)	(08) 9842 7500,	0429 804 746*
• Kimberley (Broome)	(08) 9194 1630,	0438 920 736*
• Midwest (Geraldton)	(08) 9956 1985,	0427 479 297*
• Pilbara (South Hedland)	(08) 9158 9222,	0428 637 630*
• South West (Bunbury)	(08) 9781 2350,	0429 684 236*

In the Perth metropolitan area*

• North Metropolitan	(08) 9222 8588,	0405 157 848
• South Metropolitan (Fremantle)	(08) 9431 0200,	0478 320 002
• East Metropolitan (from 01 July 2016)	TBA	TBA

* For after-hours emergency assistance, communicable disease notifications please contact the On Call Duty Officer on **(08) 9328 0553**